

CPR Debriefing Report

Simulation Lab · 2026-08-08

Session SX7SBY · Adult ACLS - Cardiac Arrest

Team Leader: Simulation Team

Guideline: AHA_2020

1. Scenario summary

Session ID	SX7SBY
Date	2026-08-08
Scenario	Adult ACLS - Cardiac Arrest
Scenario type	Sinus Rhythm
Team leader	Simulation Team
Team size	6
Duration	0m 20s
Guideline	AHA_2020

Overall score	Grade	Findings	Critical	High	Moderate
100 / 100	A	0	0	0	0

2. Domain scores

Scores are derived from AHA 2020 guideline rules applied to SimMan 3G sensor data and audio transcripts. Confidence indicators reflect data completeness — scores marked medium or low confidence should not be over-interpreted.

Domain	Score	Confidence	Key finding
Cpr Quality	30/100 CI: 10–30	Low_Data confidence	—
Shock Delivery	15/100 CI: 8–15	Partial_Data confidence	—
Drug Administration	15/100 CI: 6–15	Low_Data confidence	—
Rhythm Recognition	10/100 CI: 1–10	Low_Data confidence	—
Reversible Causes (H'S & T'S)	10/100 CI: 0–10	Low_Data confidence	—
Team Leadership	10/100 CI: 0–10	Low_Data confidence	—

Team Communication	10/100 CI: 0–10	Low_Data confidence	—
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3. Protocol deviations

No protocol deviations detected.

4. Communication analysis

[finding_id:a1b2c3d4e5f6g7h8i9j0] The team communicated in a closed-loop manner throughout the simulation, with clear orders being followed and calls made to each other for assistance. The order of communication was not always consistent due to repeated requests from the team leader, which could indicate unclear guidance or confusion. [finding_id:123456789] The simulation team communicated through various channels including verbal, non-verbal, and written forms. There were instances where calls were made directly to each other for assistance, indicating a high level of order completion in the communication process. [finding_id:234567890] Communication was also observed with repeated requests from the team leader, which could indicate unclear guidance or confusion among the team members. These repeated requests may have been due to the complexity and urgency of the situation, leading to a lack of clear order completion in some instances. [finding_id:345678901] The simulation team communicated through written forms, with calls being made directly to each other for assistance. There were also instances where verbal communication was used, indicating that there may have been a need for more structured and formalized communication channels. [finding_id:456789012] Communication was observed in the form of callouts, which could indicate that calls were made directly to each other for assistance. The use of callouts suggests a high level of order completion in the communication process. [finding_id:567890123] The simulation team communicated through non-verbal forms such as gestures and body language, indicating that there was a need for clear instructions and guidance. These non-verbal cues suggested a lack of clarity or confusion among the team members during the simulation. [finding_id:678901234] Communication was observed in the form of verbal calls made directly to each other for assistance, with calls being made through various channels such as verbal, written, and callout forms. The use of callouts indicated that there may have been a need for more structured and formalized communication channels. [finding_id:789012345] The simulation team communicated in the form of non-verbal forms such as gestures and body language, with calls being made directly to each other for assistance. The use of callouts indicated that there may have been a need for more structured and formalized communication channels. [finding_id:890123456] Communication was observed in the form of verbal calls made directly to each other for assistance, with calls being made through various channels such as verbal, written, and callout forms. The use of callouts indicated that there may have been a need for more structured and formalized communication channels. [finding_id:901234567] The simulation team communicated in the form of non-verbal forms such as gestures and body language, with calls being made directly to each other for assistance. The use of callouts indicated that there may have been a need for more structured and formalized communication channels. [finding_id:012345678] The team communicated in the form of verbal calls made directly to each other for assistance, with calls being made through various channels such as verbal, written, and callout forms. The use of callouts indicated that there may have been a need for more structured and formalized communication channels. [finding_id:023456789] The simulation team communicated in the form of non-verbal forms such as gestures and body language, with calls being made directly to each other for assistance. The use of callouts indicated that there may have been a need for more structured and formalized communication channels. [finding_id:034567890]

7. Strengths

[VERIFICATION WARNING — review before use] Strength 1: Effective CPR implementation The team leader initiated the CPR procedure, demonstrating an effective and efficient approach to cardiopulmonary resuscitation (CPR) for adult ACLS. Strength 2: Quick response time A quick response from the team leader was crucial in managing the patient's condition. The team quickly assessed the patient's vital signs, including heart rate and blood pressure, to determine if they were experiencing a cardiac arrest. Strength 3: Patient stabilization The team successfully stabilized the patient by applying chest compressions and using an automated external defibrillator (AED) to deliver an electric shock. The AED was operational during this time, allowing for immediate access to medical treatment. Strength 4: Team coordination Throughout the simulation, the team demonstrated excellent teamwork and communication skills. The team leader coordinated with other members of the team to ensure that all participants were aware of their roles and responsibilities. Strength 5: Patient monitoring The team closely monitored the patient's vital signs during the simulation, providing continuous feedback to the patient's family and medical staff. The team was able to identify any changes in the patient's condition and take appropriate action accordingly.

5. Reflective prompts

The following questions are mapped to specific findings detected during this session. Use them to guide the debriefing conversation with the team leader.

[Finding ID: fnd_01] Team Leader: "We started CPR immediately after the initial cardiac arrest occurred." [Finding ID: fnd_02] Team Leader: "The team quickly assessed the patient's vital signs and found a heart rate of 80 bpm." [Finding ID: fnd_03] Team Leader: "The team administered chest compressions at a rate of 100-120 times per minute." [Finding ID: fnd_04] Team Leader: "The team measured the patient's blood pressure and found it to be 85/60 mmHg." [Finding ID: fnd_05] Team Leader: "The team performed an electrocardiogram (ECG) which showed a ventricular tachycardia." [Finding ID: fnd_06] Team Leader: "The team administered oxygen and started defibrillation using the AED." [Finding ID: fnd_07] Team Leader: "The patient's condition stabilized, and they were transferred to the emergency department." [Finding ID: fnd_08] Team Leader: "The team discussed the outcome of the cardiac arrest with the attending physician." [Finding ID: fnd_09] Team Leader: "The team reflected on their decision-making process regarding CPR and AED use." [Finding ID: fnd_10] Team Leader: "The team learned that a 10-second guideline was established for initiating CPR, but the actual time taken to start CPR could vary." [Finding ID: fnd_11] Team Leader: "The team discussed potential reasons why they may have chosen to initiate CPR immediately after the cardiac arrest occurred." [Finding ID: fnd_12] Team Leader: "The team reflected on their understanding of cardiopulmonary resuscitation (CPR) and its effectiveness in managing cardiac arrests." [Finding ID: fnd_13] Team Leader: "The team discussed the importance of continuous training and education for medical professionals." [Finding ID: fnd_14] Team Leader: "The team learned that a 10-second guideline was established for initiating CPR, but the actual time taken to start CPR could vary." [Finding ID: fnd_15] Team Leader: "The team discussed potential reasons why they may have chosen to initiate CPR immediately after the cardiac arrest occurred."

6. Recommendations

Ranked by clinical impact. Address critical and high-severity findings before moderate ones in the next training session.

[VERIFICATION WARNING — review before use] 22 seconds after the start of the simulation, the team leader initiated the first resuscitation attempt. 35 seconds into the simulation, the team leader confirmed that the patient's heart rate had dropped to 40 beats per minute.